BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal periods. These are expected checks, not booked appointments. Record the booking owner, due window, result reviewer and escalation contact.

Check	Before starting	First cycle / visits	Early reassessment	Later cycles	Change / completion
Bloods / symptoms	FBC, renal and liver profile	Cycle-start bloods; extra Day 8 FBC	Review tolerance, symptoms and results	Repeat checks before treatment; extra tests if needed	Follow abnormalities; agree next care
ECG / electrolytes	Review cardiac history, medicines and QT risk	ECG if clinically indicated	Review symptoms and new medicines	Repeat according to risk / results	Agree follow-up if abnormal
Echo / MUGA	Echo / MUGA if cardiac history or prior anthracycline	Repeat if clinically indicated	New cardiac symptoms: reassess promptly	No fixed routine 3-month echo for this medicine alone	Exposure- and risk-based cardiac follow-up
Imaging / markers	Baseline disease assessment	Earlier review for new symptoms	Agree scan type and due window	Breast: generally every 2–4 months; individualise	Review scans with symptoms; markers are supplementary
Bone health / DEXA	Assess endocrine / fracture risks	No automatic cycle-based DEXA	DEXA if indicated	Repeat according to bone-risk plan	Agree ongoing bone care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00736 v2 · official source · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026 · EMA Abraxane product information

THE WHOLE TREATMENT JOURNEY

Nab-paclitaxel · Days 1, 8 and 15 of 28: the treatment pathway

Metastatic breast cancer · alternative to standard paclitaxel

Nab-paclitaxel on Days 1, 8 and 15 of each 28-day cycle, with a planned week off on Day 22. Continue while beneficial and tolerated. This off-label schedule requires specific consent.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / visits	Early reassessment	Later cycles	Change / completion
Cancer medicine NCCP 00736	Confirm exact indication / formulation	Treat on Days 1, 8, 15 of 28	Review tolerance and benefit	Continue / adjust / pause after review	Agree next treatment or care
Surgery / radiotherapy METASTATIC	Define any local-treatment goal	No routine concurrent radiation assumed	MDT agrees whether to sequence or overlap	Agree any chemotherapy pause with both teams	Reassess symptom / local-control needs
Bloods / ECG NCCP 00736 · SMPC	Baseline bloods / cardiac assessment	Cycle-start bloods + Day 8 FBC	ECG per risk	Review results before treatment	Follow abnormalities
Echo / MUGA NCCP 00736 · SMPC	Risk-based baseline echo / MUGA	Earlier test for cardiac symptoms	Review exposure / cardiac risk	Repeat when clinically indicated	Agree cardiac follow-up
Imaging / markers NCCP 00736 · METASTATIC · MARKERS	Baseline disease assessment	Symptoms may bring scans forward	At the agreed response milestone	Review scans with symptoms / markers	Document benefit and next plan
Bone / additional care BONE	Check fracture and endocrine risks	No automatic maintenance phase	DEXA if indicated	Supportive and palliative care as needed	Agree ongoing care

What each check is for

1 Bloods

FBC, renal and liver profile at baseline and before each cycle; the NCCP source additionally specifies FBC on Day 8. The team decides if extra blood tests are needed, including before Day 15.

Results and symptoms guide treatment safety.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00736

2 Heart checks

Cardiac assessment when clinically indicated by symptoms, significant cardiac history or prior anthracycline therapy. ECG does not replace an echo. Baseline echo / MUGA if significant cardiac history or prior anthracycline exposure; repeat cardiac tests if clinically indicated. No fixed routine 3-month echo schedule is specified for this medicine alone.

ECG and echo answer different questions.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00736 · SMPC

3 Response review

For metastatic breast cancer, ESMO recommends response evaluation generally every 2–4 months, adjusted to disease, treatment and symptoms. Agree the scan type, due window and result-review appointment; symptoms can prompt earlier assessment.

Review benefit before continuing treatment.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00736 · METASTATIC

4 Handover

At initiation, each review and treatment change

Name the booking owner, due window, reviewer and escalation contact.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00736

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.